

Urination Pain - Female - Female (Adult)

Algorithm 1481 | Adult (18 years and older) | Gender at birth: Female

Definition

- UAT-only adult and pediatric pathway for dysuria in patients with female urinary/reproductive anatomy, distinguishing sepsis, pyelonephritis, retention/obstruction, pregnancy/postpartum and genital or STI-related causes from possible lower UTI.
- Adult patient.
- Female.

Initial assessment

1. [OPEN_TEXT] Describe burning or pain, frequency, urgency, urine amount/flow, blood/cloudiness/odor, lower abdominal pain, and when symptoms began.
2. [TEMPERATURE] What is the temperature if measured, and are there rigors, flank/back pain, nausea, vomiting, confusion, weakness or reduced urine?
3. [YES_NO] Is the patient pregnant, possibly pregnant, within 6 weeks after birth, diabetic, immunocompromised, catheterized, or known to have kidney/urinary abnormalities?
4. [YES_NO] Is there inability to urinate, painful bladder fullness, severe one-sided pain, recent urinary/pelvic trauma or procedure, or catheter blockage/dislodgement?
5. [YES_NO] Is there vaginal/vulval pain, itching, sores, discharge, bleeding, pelvic pain, pain during sex, or possible STI exposure?
6. [YES_NO] For a child or adolescent, is there fever, vomiting, poor feeding, reduced wet nappies/urine, new wetting, abdominal/back pain, genital symptoms, or any privacy or safeguarding concern?

Triage and redirect questions

1. Redirect → Flank Pain

Is pain in the side or back the main concern rather than pain primarily during urination?

Pain focused in the side or back belongs in the Flank Pain pathway.

1. Disposition level 100

Along with urinary symptoms, is there confusion, difficult waking, fainting, fast or difficult breathing, mottled/pale skin, severe weakness, very high or low temperature, inability to keep fluids down, very low urine output, severe or rapidly worsening pain, or a seriously unwell infant or child?

These may indicate sepsis, severe pyelonephritis, acute kidney injury or another time-critical illness. Temperature alone neither confirms nor excludes sepsis.

1. Disposition level 70

Is there fever/rigors, flank or back pain, vomiting, blood in urine, inability or marked difficulty urinating, pregnancy/possible pregnancy or recent birth, age under 16 or 65+, catheter, diabetes/immunosuppression, urinary abnormality, recurrent symptoms, genital discharge/sores/pelvic pain, or no improvement within 48 hours of clinician-prescribed treatment?

These features require same-day in-person assessment for upper/complicated UTI, retention/obstruction, pregnancy complications or another diagnosis. Children require age-specific urine testing; genital symptoms may require confidential STI/reproductive assessment rather than empirical UTI treatment.

1. Disposition level 70

For a non-pregnant adult with mild isolated dysuria and no red flag, has urine testing or an approved Qatar lower-UTI assessment pathway not yet confirmed that self-care or delayed prescribing is appropriate?

Dysuria is not specific to cystitis. This UAT telephone pathway does not prescribe antibiotics, set unsafe delay thresholds, or exclude pregnancy, pyelonephritis, STI, vaginitis, obstruction or other causes without in-person testing and local governance.

Advice

Qatar urinary emergency response

- Call Qatar emergency services on 999 and do not allow self-driving. Keep the person safely supervised. Do not force oral fluids if drowsy, vomiting, unable to swallow, unable to urinate or awaiting emergency assessment. Do not use leftover antibiotics or another person's medicine. Call Qatar 999 now, follow the call-taker's ambulance and emergency-transport instructions, and do not self-drive. Do not delay emergency care.
- Call Back If: worsening confusion; breathing difficulty; collapse; very low urine; worsening pain.

Same-day urinary and reproductive assessment

- Arrange same-day in-person assessment through an approved Qatar adult, maternity or pediatric pathway for urine sampling before antibiotics when feasible and clinician-directed examination/testing. Pregnancy or recent birth, fever/flank pain, retention, blood, catheter problems and genital symptoms require tailored review. Adolescents should be offered developmentally appropriate private time, within Qatar consent, confidentiality and safeguarding policy.
- Call Back If: fever or rigors; back or flank pain; vomiting; cannot urinate; blood in urine; becomes very unwell.

Testing before a lower-UTI plan

- Until assessed, rest and drink normally to thirst if fully alert and not vomiting or fluid-restricted. Do not force excessive fluids, start leftover antibiotics, use urinary alkalinizing products, or give fixed-dose/timed analgesia unless an approved Qatar clinician or protocol confirms age, weight, pregnancy, allergies, kidney function, interactions and maximum dose. The exact non-emergency destination and medicine pathway are local approval required.
- Call Back If: symptoms worsen; fever or flank pain; vomiting; reduced urine; pregnancy concern.

First aid

- Call Qatar emergency services on 999 and do not allow self-driving. Keep the person safely supervised. Do not force oral fluids if drowsy, vomiting, unable to swallow, unable to urinate or awaiting emergency assessment. Do not use leftover antibiotics or another person's medicine. Call Qatar 999 now, follow the call-taker's ambulance and emergency-transport instructions, and do not self-drive. Do not delay emergency care.

References

- Qatar Ministry of Public Health, "Healthcare Services in Qatar", <https://sportandhealth.moph.gov.qa/EN/faninfo/Pages/HealthcareServicesInQatar.aspx> (accessed 2026-07-25)
- NICE NG109, "Urinary tract infection (lower): antimicrobial prescribing" (updated 2026)
- NICE NG111, "Pyelonephritis (acute): antimicrobial prescribing" (accessed 2026-07-25)
- CDC, "Adolescents - STI Treatment Guidelines" (accessed 2026-07-25)

Provenance

Shaped to mirror the licensed After-Hours Telehealth Triage Guidelines database structure. This is IST open-source guideline content derived from cited public sources; it is not licensed STCC content. Question.DispositionLevel=null marks a See More Appropriate Guideline redirect. Demographic variants contain only source-supported differences; identical thresholds are documented rather than fabricated. Local destinations and TelemedicineEligible values require IST clinical governance approval. UAT DATA ONLY - NOT FOR REAL-PATIENT CARE OR PRODUCTION USE.

Manifest hash: 4b1dd76d9a541d1f533025b2f2fc597a289cf4ea6ecdb499bda0ce9aaa94624c | Status: BLOCKED_BY_RESEARCH_GAP